INDEPENDENT RESEARCH REVIEW

The Federal "Treatment First" Homelessness Toolkit

Critical analysis, connected research, federal funding implications and final findings

Review of SAMHSA publication PEP26-04-009 (August 2026)

Research synthesis prepared October 15, 2026

Scope: full 40-page toolkit • 41 cited references • selected independent evidence and federal program documents

An evidence-based critique. No allegation of corruption is treated as proven without supporting records.

Executive assessment

Core conclusion: The toolkit makes a strong case for accessible, evidence-based addiction treatment, but it does not establish that requiring treatment as a condition of housing produces better overall homelessness outcomes than Housing First paired with adequate support services. Key evidence is selectively framed, economic causes are underdeveloped, and several implementation safeguards are insufficiently specified. [1, 3–8]

The report examines the document as a federal policy proposal, not as a clinical guide alone. Its claims are tested against the cited literature, outside peer-reviewed research, the program notices implementing elements of the philosophy, and questions about real-world accountability.

Priority finding	Evidence-based assessment
Housing First evidence	Peer-reviewed synthesis shows substantially improved housing stability; the toolkit gives this evidence little weight. [4]
Veterans overdose study	A real, troubling association is used to support a broader policy conclusion than the observational study can prove. [6]
Treatment conditions	A federal STREETS grant expressly conditions housing connection on treatment participation. [3]
Outcome metrics	Employment, abstinence, and unsubsidized housing are elevated above individual variation in ability and support needs. [1]
Encampment clearance	Sites may be cleared even when some individuals are not immediately housed. [1, pp. 25–26]
Funding changes	HUD and SAMHSA implementation has concrete grant and legal consequences. [13–16]

How to read the conclusions

Documented: established by explicit toolkit language or verified source documents. **Research-based concern:** an interpretation supported by evidence but not a finding that a particular agency acted illegally. **Unverified:** facts requiring underlying records, such as claims of conflicts of interest, improperly selected beneficiaries, or intentionally manipulated research.

Scope limitation: The toolkit has 41 references. This report checks the most consequential research connections and operational claims, but is not a line-by-line reproduction or forensic validation of every dataset. The original toolkit is the primary source for its own policy claims. Its printed page numbering starts on PDF page 4; this report cites the PDF page number unless marked otherwise. [1]

1. Treatment-linked housing is a real funding requirement

Toolkit PDF page 24 • STREETS grant SM-26-019

The toolkit identifies the Safety Through Recovery, Engagement, and Evidence-Based Treatment and Supports (STREETS) initiative as a funding opportunity and states that housing assistance requires treatment participation. The underlying SAMHSA grant notice is unusually explicit: connections to housing supported by the grant are contingent on treatment participation, and housing pathways must use treatment-engagement and compliance criteria. [1, 3]

This distinction is central. **Offering treatment to a housed person** and **requiring treatment before a housing placement** are different interventions with different consequences for access. STREETS supports evidence-based treatment and explicitly protects the use of FDA-approved addiction and psychiatric medications in housing placements, but it imposes an entry condition that the toolkit does not demonstrate improves housing retention. [3]

Important limit: STREETS is a particular grant, not evidence that all federally funded housing programs now share this condition. Nevertheless, it is direct evidence that treatment-conditional housing is being built into policy implementation.

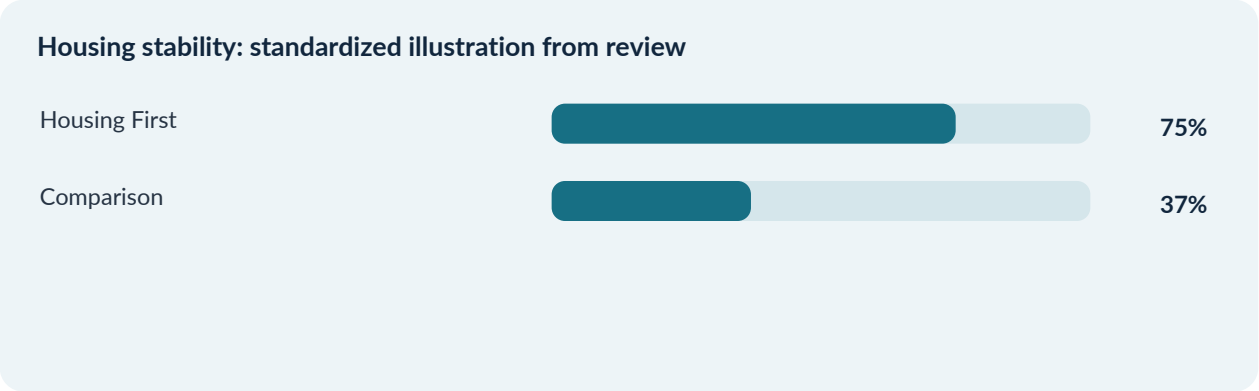
> *Testable question: Are treatment-conditional placements more likely to achieve safe, durable housing outcomes than low-barrier placements offering the same clinical services?*

2. Housing First is criticized without adequate engagement with its strongest evidence

Toolkit PDF pages 5–6

The toolkit argues that Housing First and “Housing Only” fail to prevent overdoses, expand treatment, or establish self-sufficiency. Housing First, however, is not synonymous with no treatment: its defining feature is rapid access to permanent housing without sobriety or treatment as an entry precondition, with supportive services offered alongside housing. [1, 4]

A 2019 systematic review of randomized trials found that Housing First participants were more likely to be stably housed after roughly 18–24 months (risk ratio 2.46; 95% CI 1.58–3.84). It also found lower emergency-department use (IRR 0.63) and hospitalization (IRR 0.76). The review found no clear difference in problematic substance use, and all four included studies were assessed as high risk of bias. Those limits must be acknowledged alongside the beneficial housing outcomes. [4]



The illustrated 75% and 37% housing-stability figures come from the review's evidence table, rather than a single national probability applicable to every program. The stronger and more portable effect estimate is the pooled relative risk of 2.46. [4]

Assessment: It is supportable to say Housing First does not reliably cure addiction or serious mental illness. It is not supportable to treat those limitations as evidence that it failed at its central housing-stability objective. The toolkit's sweeping conclusion is broader than this research supports.

3. The cited veterans overdose study supports improving treatment, not proof that housing is harmful

Toolkit PDF page 5 • toolkit reference 8

Tsai, Peltzman and Beydoun studied 60,888 veterans entering HUD-VASH supportive housing and a propensity-score-matched group of veterans experiencing homelessness. The observational study reported approximately twice the overdose mortality risk among supported-housing participants across the measured follow-up points. This is a serious warning about the need for overdose prevention and effective treatment. [6]

But propensity matching does not remove every difference in clinical severity, underlying risk, or selection into housing. It cannot by itself prove that housing caused the mortality difference. The study authors called for timely substance-use treatment *within* supportive housing. The toolkit moves from an association to a system-wide rationale for Treatment First without demonstrating the superiority of mandatory treatment preconditions. [6]

A complementary 2025 analysis of the randomized At Home/Chez Soi Housing First trial evaluated mortality in 2,255 participants across five Canadian cities. The adjusted hazard ratio was 0.83 (95% CI 0.43–1.22), with no statistically significant mortality difference. This neither proves Housing First reduces mortality nor supports a claim that it increases mortality. [7]

Finding: The observational study is relevant and troubling, but the most defensible interpretation is a need to improve addiction treatment and overdose prevention in housing, not proof that housing must be withheld until a person accepts treatment.

4. Independent modeling also supports evaluating housing without treatment prerequisites

In a 2025 JAMA Network Open economic model, researchers estimated five-year outcomes for 1,000 unhoused adults with opioid use disorder who were offered stable housing without a treatment-entry requirement. The model estimated 191 deaths under the status quo and 140 under a stable-housing intervention, a difference of 51 deaths, at an estimated incremental cost of $26,800 per quality-adjusted life-year gained. [5]



These are modeled estimates dependent on assumptions; they are **not** observed trial results. They also do not establish that housing alone eliminates overdose risk. Their importance here is that a directly relevant comparison supports studying non-conditional housing, yet this literature is largely missing from the toolkit's advocacy for conditional pathways. [5]

5. National prevalence figures are presented without adequate methodological context

White House August 12 release • toolkit introduction

The accompanying White House release asserts that 75% of homeless Americans are addicted to drugs and 78% have mental illness. The announcement does not adequately state the source population, definitions, sampling method, or diagnostic threshold that produced those percentages. [2]

KFF, analyzing HUD's 2025 Point-in-Time data, reported that approximately 18% of adults experiencing homelessness met HUD's substance-use-disorder definition and 24% met its serious-mental-illness definition. These are narrower constructs than "drug addiction" and "mental illness," so the percentages are **not directly equivalent**. HUD measures may undercount less severe conditions. The White House claim remains insufficiently documented rather than definitively mathematically disproved. [8]

The toolkit prominently prints a combined count of 1,456,923 people who were homeless *or living in taxpayer-funded housing for people who had experienced homelessness*. HUD's actual 2025 PIT homelessness count was 745,652, including 266,320 unsheltered people; HUD states that people in taxpayer-funded housing assistance are not included in the PIT count. The combined number is not necessarily an arithmetic error but is prone to misleading interpretation if the distinct categories are obscured. [1, 9]

Finding: The scientific and public-policy case requires clearly defined denominators and consistent diagnostic categories. Public messaging should not imply that every person receiving a homelessness-related housing subsidy is still homeless.

6. Housing affordability is underweighted as a cause of homelessness

The toolkit devotes sustained attention to addiction, trauma, mental illness, employment and personal recovery while offering much less analysis of rent burdens, housing supply, local affordability, and access to permanent homes. Its illustrative route toward self-sufficiency culminates in market-rate or unsubsidized housing, without showing that participants can afford available housing in their communities. [1]

The U.S. Government Accountability Office found that a $100 increase in median rent was associated with about a 9% increase in estimated homelessness in studied areas, controlling for other factors. This is an observational association, not a controlled experiment. [10]

UCSF's California statewide study reported a median household income of $960 per month in the six months before homelessness among surveyed respondents, and substantial perceived potential benefits of rent subsidies or one-time financial help. [11]

Finding: Treatment, recovery and employment cannot alone guarantee that a person can obtain a home when local rents exceed income. A national response requires an explicit housing-affordability strategy alongside healthcare.

7. Encampment clearance is treated as resolution without guaranteed rehousing

Toolkit PDF pages 25–26 • Chapter 5

The five-step HOST framework calls for identification, needs assessment, repeated outreach, posting deadlines, and "encampment resolution." Step three allows outreach to end after all reasonable engagement efforts; step five permits service linkage for people who are not immediately housed, while the site is cleared and restored. [1]

The toolkit reports more than 1,500 encampments resolved with no arrests, use of force, or litigation. Those are consequential process claims, but the chapter does not provide an independent dataset establishing how many people became and remained permanently housed. The number of locations cleared should not substitute for person-level housing outcomes. [1]

Especially revealing, toolkit reference 28 is an issue brief from the National Health Care for the Homeless Council. That source says forced encampment sweeps can damage health and care connections, compromise safety and civic trust, undermine housing pathways, and create costs. The toolkit uses sources that describe encampment hazards but does not comparably explore harms from clearance itself. [1, 12]

Finding: There is a meaningful distinction between resolving a location and resolving the homelessness of the people living there. Programs should publish placements, subsequent housing retention, relocations to other encampments, and treatment continuity, not solely cleared-site counts.

> *Accountability test: How many residents were in stable housing 6 and 12 months after the encampment was cleared?*

8. Performance measures may create incentives to exclude harder-to-serve participants

Toolkit PDF pages 11–15 • Chapter 2

The toolkit proposes useful measures such as costs per client, warm handoffs, service engagement, housing retention and returns to homelessness. Yet its “thriving” profile prominently emphasizes long periods of abstinence, full-time employment above minimum wage, employer-provided benefits, and unsubsidized housing. [1]

These outcomes may be appropriate for some participants, but not universal definitions of success. Older adults, people with disabilities or serious chronic illnesses, caregivers, and individuals whose work capacity is limited may remain stably housed and meaningfully involved in their communities without full-time employment or independence from subsidies.

If performance payments reward low unit costs, treatment graduation, abstinence and market-rate housing without meaningful adjustment for participant needs, providers can be incentivized to admit easier cases. Comparisons should report all program entrants (including exits and failures), stratify results by baseline need, and measure retention at 6, 12 and 24 months. [1]

Finding: A system can be more accountable while still being more inclusive. Stable housing, safety, dignity, clinically relevant progress, and participant-defined quality of life deserve equal standing with employment and abstinence metrics.

9. The document's own principles are in unresolved tension

Stated principle	Implementation tension
No Wrong Door	Housing connection can be contingent on treatment participation in some funding pathways.
Individualized care	Full-time work and unsubsidized housing are prominent late-stage benchmarks.
Trauma-informed care	Encampment removal can proceed without permanent placement for every resident.
Recovery is nonlinear	Substance-free living rules can trigger a housing transfer upon relapse; uninterrupted access is not guaranteed in the text.
Voluntary spiritual support	Strong preference for faith-based partners requires clear and accessible secular alternatives.
Transparency	Featured programs and clearance counts do not come with standardized independent comparisons.

These are policy tensions and incomplete implementation guidance, not proof of a direct logical contradiction or unlawful administration. They call for written safeguards before broad replication. [1, 3]

10. Faith-based approaches are promising for some people but not universal requirements

The toolkit appropriately recognizes long-standing faith-based service networks and states that religious belief should not be imposed. Some cited research finds associations between spirituality and reduced harmful substance use; these associations must not be mistaken for evidence that mandatory religious instruction improves clinical outcomes. [1, references 17–21]

A defensible public program should preserve participant choice, secular treatment options, clinical standards, nondiscrimination, and clear separation of voluntary religious activities from treatment or housing eligibility. The toolkit supports inclusion of both faith-based and secular providers but does not detail comparable safeguards for all program settings.

Finding: Collaboration with religious organizations may be valuable. It does not exempt programs from outcome evaluation or establish that faith-centered services are superior to secular evidence-based care.

11. Work therapy, child screening and electronic monitoring require implementation safeguards

Toolkit PDF pages 10, 18, 28–34

The document endorses meaningful activities, chore schedules, work therapy, social enterprises and workforce pathways. These can build skills, but any work yielding material operational or commercial benefit requires careful analysis under applicable wage-and-hour rules. Merely calling an activity “therapeutic” does not determine its legal status.

Chapter 6 promotes broad identification of drug-endangered children, screening in child-facing service settings, and multidisciplinary coordination, while also supporting family unity where safe. Safeguards must address accurate screening, confidentiality, unnecessary family separation, due process, and the difference between risk markers and evidence of actual abuse or neglect. [1]

Chapter 7 explicitly describes AI-enabled services, electronic ID-card monitoring of clients’ daily residential activities, and tracking clients throughout a county healthcare system. These proposals warrant patient-consent controls, data minimization, retention limits, oversight, and compliance with HIPAA and applicable substance-use confidentiality protections, including 42 CFR Part 2 where appropriate. The toolkit mentions confidentiality guidance but lacks detailed implementation rules. [1]

Finding: The most credible critique is insufficient detail about safeguards; this document alone does not establish privacy, labor, or civil-rights violations by any named provider.

12. The policy is linked to actual federal funding and ongoing legal disputes

SAMHSA announced $42.3 million in supplemental block-grant funding in September 2026 to advance Treatment First principles: $17.3 million through mental-health block grants to build system capacity and $25 million through substance-use block grants to expand recovery-housing capacity. It separately announced $2 million for addiction technology transfer-center support. These amounts have distinct purposes and should not be conflated. [13]

HUD’s FY2026 Continuum of Care funding opportunity exceeds $4 billion and includes a contested $1.3 billion set-aside associated with transitional housing and supportive-services-only projects. The district court vacated the notice, but the First Circuit stayed that order on September 16, 2026 pending appeal. HUD reopened the competition; the legal merits were not definitively resolved by the stay. [14, 15]

On September 11, 2026 HUD also published a Federal Register notice presenting research to justify additional incentives for certain homelessness interventions. That notice is particularly important because it seeks to connect studies with funding preferences under the statutory “proven effective” framework. [16]

Finding: The debate is consequential for grants and access to assistance. The strongest documentary question is whether research establishes better *housing outcomes* for conditional pathways, not simply whether treatment programs can produce recovery outcomes among participants who enroll.

Chapter-by-chapter audit

Chapter / PDF pages	Strengths	Critical gap
Introduction **4–7**	Recognizes serious public-health needs	Frames Housing First as failed without balanced evidence review.
1. Core elements **8–10**	Trauma-informed care, peer support, warm handoffs	Access restrictions and structure may conflict with individual choice.
2. Outcomes **11–15**	Long-term measurement and retention indicators	Abstinence and labor-market benchmarks may not fit every person.
3. Continuum **16–20**	Care transitions, asset mapping and partnerships	Housing framed as progression through recovery stages.
4. Financing **21–24**	Identifies financing pathways	Includes treatment-conditional housing funding.
5. Encampments **25–27**	Outreach and stated no-arrest model	Site clearance allowed before full rehousing.
6. Children **28–30**	Child safety and family preservation	Screening and interagency coordination need stronger safeguards.
7. Looking ahead **31–34**	Evidence improvement and telehealth	Monitoring/privacy and treatment model validation unresolved.
References **35–37**	41 bibliographic references	Key Housing First evidence omitted or narrowly interpreted.
Appendices **38–40**	Contributors and 15 featured programs identified	Independent comparable program outcomes not supplied.

Research-integrity assessment

Key evidence link	What can fairly be concluded
Toolkit ref. 8: veterans mortality	Overdose risk association is real; causal effect of housing is not demonstrated. [6]
Housing First trial meta-analysis	Substantial housing-stability improvement; limited and mixed health effects. [4]
Randomized mortality follow-up	No significant mortality benefit or harm demonstrated. [7]
Housing for OUD economic model	Potential mortality and cost-effectiveness gains; modeled, not experimental. [5]
PIT count vs funded housing	Different populations combined in a headline figure. [9]
White House 75% / 78% figures	Source definitions insufficiently presented; not directly comparable to HUD PIT definitions. [2, 8]
Toolkit ref. 28: encampment sweeps	Source provides important counterevidence on clearance harms. [12]
Featured programs, Appendix B	Need standardized admission, retention and independent outcome data. [1]

Implications for local accountability: a Chico test

The research does not establish new facts about particular Chico agencies or shelters. Instead, it offers a common audit standard for any locally funded program, regardless of whether that organization identifies as Housing First, recovery-centered, transitional, or emergency shelter.

Question	Required evidence
Who is actually housed?	Unique people entering permanent housing and percentage of all eligible participants.
Who stays housed?	Housing retention at 6, 12 and 24 months, including returns to homelessness.
Who is excluded?	Denials, eligibility restrictions, program terminations and appeals.
What happens after a relapse?	Continuity of care, safe alternatives, and whether housing is preserved.
What happens after a sweep?	Housing destinations and housing status of all displaced residents.
What do funds accomplish?	Cost per durable housing outcome, adjusted for support needs.

A shelter can serve thousands of meals while producing comparatively few permanent housing exits; conversely, a housing program can keep residents stably housed while failing to meet serious clinical needs. Both should be accountable for what they promise to achieve.

Unresolved questions: an appropriate next-stage records investigation

The appendices disclose summit participants and 15 programs presented as examples. The presence of contributors from featured organizations is not itself misconduct. To determine whether improper influence or conflicts exist, investigators would need concrete records rather than inferences from overlapping names. [1]

- **Contracting and authorship:** procurement files, author selection, scientific review procedures, drafts and editorial changes.
- **Financial relationships:** relevant disclosures, grant awards, subcontracts and financial ties involving featured programs and decision-makers.
- **Independent performance:** enrollment, refusals, terminations, deaths, 6–24-month housing retention, treatment outcomes and comparable costs.
- **Research selection:** memoranda documenting inclusion/exclusion of Housing First trials and interpretation of the veterans mortality finding.
- **Local implementation:** how grant conditions affect shelter eligibility and service availability in each participating community.

Assessment of corruption: Evidence examined here does not establish bribery, fraud, intentional falsification, illegal steering of funds, or unlawful conflicts. Selective research framing and policy overreach are defensible criticisms; a finding of misconduct requires additional proof.

Final findings and public-facing conclusion

1. Housing First has substantial evidence for housing retention. Its benefits for substance use, mental health and mortality are less uniform. Recognizing those limits supports more effective services, not the inference that stable housing must await treatment. [4, 6, 7]

2. Treatment First is not proven superior as a homelessness strategy. The toolkit brings legitimate treatment expertise but lacks a rigorous comparison showing that its conditional pathways house more people, keep them housed longer, or improve overall outcomes. [1, 3–7]

3. Program goals and population needs should not be conflated. People may need affordable housing without addiction treatment, or continued subsidies without full-time employment; those facts do not make their housing outcomes failures. [1, 8, 10, 11]

4. Site clearance and service utilization cannot substitute for ending homelessness. Outcome reporting should follow people over time, include those rejected or discharged, and measure displacement effects. [1, 12]

5. Federal spending and conditions demand public scrutiny. With large grant programs underway and judicial review in progress, research standards, civil rights, patient choice and transparent housing outcomes are immediately consequential. [3, 13–16]

> *Central challenge: If treatment services within Housing First are inadequate, why is replacing Housing First presumed to be better than fixing the services? What comparative evidence supports requiring treatment before stable housing?*

Final assessment: This is a serious, evidence-based critique of selective scientific framing and the potential harms of treatment-conditional housing. The strongest currently documented claim is **not corruption**; it is that a major federal policy shift has not yet been supported by convincing comparative evidence demonstrating better homelessness outcomes. That question deserves a transparent answer before the model is treated as a proven replacement.

Source notes and linked bibliography

Bracketed numbers refer to sources below. For statements about what the toolkit recommends, source [1] is the uploaded original. Other entries link to research, public records, and government notices; the titles are clickable in PDF readers. Each source was selected for its direct relevance to a claim in this report.

[1] **Original federal toolkit (primary source)**. SAMHSA, Best Practices Toolkit: Addressing Homelessness and Addiction through "Treatment First," publication PEP26-04-009, August 2026. User-provided 40-page PDF.

[2] **Official White House release**. ONDCP, HHS, and HUD launch first-ever Treatment First toolkit, August 12, 2026.

[3] **STREETS federal grant notice**. SAMHSA, Safety Through Recovery, Engagement, and Evidence-Based Treatment and Support, SM-26-019 (June 2026), including housing participation requirements.

[4] **Housing First randomized-trial review**. Baxter AJ et al., Effects of Housing First approaches on health and well-being, Journal of Epidemiology and Community Health (2019).

[5] **Housing and opioid-use-disorder model**. Rao I, Brandeau ML, Modeling Health and Economic Outcomes of Providing Stable Housing to Homeless Adults With OUD, JAMA Network Open (2025).

[6] **Veterans mortality observational study**. Tsai J, Peltzman T, Beydoun HA, Changes in All-Cause, Overdose, and Suicide Mortality Risk in the First 2 Years of Supported Housing, American Journal of Public Health (2026).

[7] **Randomized Housing First mortality follow-up**. Housing and Support Intervention and Mortality Among Homeless Adults With Mental Illnesses, JAMA Network Open (2025), secondary analysis of At Home/Chez Soi trial.

[8] **KFF homelessness and mental-health analysis**. KFF, Five Key Facts About People Experiencing Homelessness (2026), drawing on HUD 2025 PIT data.

[9] **HUD 2025 homelessness count**. HUD, 2025 Annual Homelessness Assessment Report Part 1 release, May 29, 2026.

[10] **GAO rental cost association**. U.S. Government Accountability Office, GAO-20-433, Homelessness: Better HUD Oversight of Data Collection Could Improve Estimates (2020).

[11] **California homelessness research**. UCSF, California Statewide Study Investigates Causes and Impacts of Homelessness (June 2023).

[12] **Encampment sweep consequences**. National Health Care for the Homeless Council, Issue Brief: Impact of Encampment Sweeps on People Experiencing Homelessness (December 2022).

[13] **Treatment First implementation funding**. SAMHSA, $42.3 million in supplemental Treatment First awards and $2 million for technical assistance (September 18, 2026).

[14] **HUD 2026 Continuum of Care competition**. HUD, FY2026 CoC program competition and litigation status, updated September 2026.

[15] **Federal appeal and procedural status**. First Circuit, National Alliance to End Homelessness v. HUD, stay order, September 16, 2026.

[16] **Federal Register research notice**. HUD, Notice of Research Justifying Additional Incentives for Certain Activities to Reduce Homelessness, 91 FR 57901 (September 11, 2026).

Research limitation: This report is an independent synthesis and is not a legal opinion, proof of criminal misconduct, or a complete independent performance audit of all featured providers. Status and grant conditions are described as of October 15, 2026.